

1. Administrative & Study Identification

Full Study Title: Real-world study assessing Efficacy of TezepeLumaB in patients with severe asthma regardless of phenotype in Russia - ELBRUS
Short Title/Acronym: ELBRUS Study
Protocol Number: D5180R00029
NCT Number: NCT06566885
Sponsor Name: AstraZeneca
Investigational Product: Tezepelumab
Phase of Development: Observational / N/A (Phase IV equivalent Real-World Study)
Medical Monitor: AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess patient-reported outcomes (specifically the Asthma Control Questionnaire, ACQ-5) after tezepelumab treatment initiation in participants with severe asthma in real-world settings. **Secondary Objectives:** To evaluate changes in SNOT-22 (Sino-Nasal Outcome Test) scores, severe asthma exacerbations, medication use (including background asthma therapy), and healthcare resource utilization.

2.2 Background & Rationale To address the need for real-world evidence (RWE) on the effectiveness of Tezepelumab in a broad population of patients with severe asthma in Russia. Tezepelumab is indicated as an add-on maintenance treatment for severe asthma regardless of phenotype. This post-reimbursement study will assess how treatment impacts patient-reported outcomes, exacerbation rates, and overall healthcare utilization outside of strictly controlled clinical trials.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (multi-centre, retrospective-prospective, non-interventional cohort study)
Intervention Model: N/A **Control Type:** Single-arm (Non-comparative) **Blinding / Masking:** Open-label / Unblinded / None
Allocation: N/A **Randomization:** N/A **Primary Purpose:** N/A **Endpoint Classification:** N/A

3.2 Planned Enrollment & Duration Target Enrollment (\$N\$): 125 adult and adolescent patients (Actual Enrollment: 125) **Healthy Volunteers:** No **Number of Sites:** Approximately 20 sites in Russia
Estimated Study Start Date: 10-Sep-2024 **Estimated Primary**

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female patients aged ≥ 18 years (and eligible adolescents ≥ 12 years). 2. Prescribed and initiated treatment with tezepelumab according to the SmPC and local Russian market reimbursement criteria (enrolment must occur within 4 weeks of the first dose). 3. Diagnosis of asthma established for at least 52 weeks prior to tezepelumab initiation and symptoms confirmed by the Investigator not to be due to alternative diagnoses. 4. Received at least one prescription of medium or high doses of inhaled corticosteroids (ICS) during the 52 weeks prior to tezepelumab initiation (defined by GINA 2022).

4.2 Exclusion Criteria 1. Any contraindication to tezepelumab as per the approved product SmPC in Russia or in the opinion of the Investigator. 2. Administration of a concurrent biologic drug for asthma since the index date (except for stable allergen immunotherapy defined as a stable dose and regimen at the time of enrolment). 3. Switched from other biologic(s) to tezepelumab with a timeframe of < 30 days since the last prior asthma biologic drug. The number of participants with prior biologic treatment (switching to tezepelumab) should be targeted at 20% or less.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Monthly administration of tezepelumab per standard of care and local reimbursement policies. **Route of Administration:** Subcutaneous (SC) injection. **Duration of Treatment:** 52 weeks (12 months) of prospective observational follow-up.

5.2 Concomitant & Prohibited Therapy Permitted: Background asthma therapy (e.g., ICS, LABA) and stable allergen immunotherapy (must be a stable dose and regimen at enrolment). **Prohibited:** Concurrent administration of other biologic drugs for asthma.

6. Study Timeline & Visit Schedule

| Visit ID | Window | Key Activities/Procedures | | :--- | :---
| :--- | | **Baseline (Retrospective)** | -52 Weeks to Day 0 |
Collection of historical data, severe asthma exacerbations, historical PROs, and medication use prior to index date. | |
Index Date | Day 0 | Tezepelumab treatment initiation, Enrolment (up to 4 weeks post-dose). | | **Interim Follow-ups** | Weeks 4, 12, and 24 | Prospective data collection in routine clinical practice: ACQ-5, SNOT-22, exacerbations, healthcare utilization.

| | **End of Study** | Week 52 | Final PRO collection, safety outcomes, and overall 1-year treatment assessment. |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Change in patient-reported outcomes using the Asthma Control Questionnaire (ACQ-5) score from baseline to 52 weeks. **Measurement Method:** Patient-completed validated questionnaire collected prospectively at designated standard-of-care visits.

7.2 Secondary & Exploratory Endpoints - Change from baseline in Sino-Nasal Outcome Test (SNOT-22) score at Weeks 4, 12, 24, and 52. - Annualized rate of severe asthma exacerbations. - Changes in healthcare resource utilization and concomitant asthma medication use (e.g., ICS sparing effect).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine real-world pharmacovigilance data collection during standard clinical visits. **Serious Adverse Events (SAEs):** Reported to the sponsor and local regulatory agencies within the standard observational timeframe (typically 24 hours of site awareness). **Data Monitoring Committee (DMC):** N/A for an observational/post-market real-world study.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All enrolled patients who meet eligibility criteria, receive ≥ 1 dose of tezepelumab, and have valid baseline and follow-up data. **Sample Size Justification:** N=125 is adequately powered to observe meaningful shifts in ACQ-5 scores and evaluate real-world practice patterns in the Russian clinical setting. **Handling of Missing Data:** As an observational study, missing data will largely be analyzed "as observed" without imputation, utilizing mixed-effect models for repeated measures (MMRM) where appropriate.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Pharmacovigilance Practices (GPP), the Declaration of Helsinki, and applicable Russian regulatory requirements.

Monitoring Plan: Source data verification and centralized data monitoring based on the observational study plan. **Audit Trail:** Electronic Case Report Form (eCRF) utilizing an EDC system with comprehensive audit trails and data clarification mechanisms.

11. Study Identifiers & Governance

Primary ID: D5180R00029 **Registry Number:** NCT06566885 **EudraCT Identifier:** N/A **CTIS Identifier:** N/A **Sponsor/Lead Organization:** AstraZeneca **Verification Date:** 01-Feb-2026 **Study Title:** ELBRUS **Official Regulatory Title:** Real-world study assessing Efficacy of TezepeLumaB in patients with severe asthma regardless of phenotype in Russia - ELBRUS

12. Clinical Framework (Severe Asthma Specific)

Primary Condition: Severe Uncontrolled Asthma **Diagnosis Threshold:** Established asthma diagnosis $\geq$ 52 weeks requiring medium/high dose ICS **Medical Methodology:** Add-on maintenance biologic therapy within real-world settings **Optimization Target:** Improvement in Asthma Control (ACQ-5) and reduction in exacerbations

13. Study Procedures & Methodology

Management Pathway: Standard routine clinical care in Russian respiratory centers **Education Protocol (HCP):** SmPC-guided prescribing and real-world data collection training **Education Protocol (Patient):** Standard disease education and PRO questionnaire (ACQ-5, SNOT-22) self-completion guidance **Quality Audit Mechanism:** Periodic source data verification against patient medical charts

14. Observation & Follow-Up Schedule

Total Duration: 52 weeks prospective follow-up (plus 52-week retrospective baseline) **Onsite Visit Cadence:** Index date, and alignment with routine care around Weeks 4, 12, 24, and 52 **Remote Monitoring Frequency:** Periodic centralized EDC data review **Checklist Review Interval:** Data entry and edit checks performed corresponding to the specified assessment intervals

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				